

Capivasertib with fulvestrant

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What is capivasertib (Truqap®) with fulvestrant?

Capivasertib with fulvestrant is a combination of cancer drugs. It is used to treat [breast cancer](#).

This treatment is used when tests show that the cancer is oestrogen receptor positive and HER2 negative. The cancer also has one or more of the following [genetic changes](#) in the cells:

- PIK3CA
- AKT1
- PTEN.

This treatment may be used if the cancer has either:

- spread to the tissues and lymph nodes close to the breast ([locally advanced](#)) and surgery is not an option, or
- spread from the breast to other parts of the body ([metastatic or secondary breast cancer](#)).

Capivasertib is also called Truqap[®]. It is a type of targeted therapy drug called an AKT inhibitor. Capivasertib blocks the action of proteins called AKT kinases which help cancer cells to grow and spread.

[Fulvestrant](#) is a hormonal therapy drug. The hormone oestrogen can encourage some breast cancers to grow. These breast cancers have receptors (proteins) on the cancer cells which allow oestrogen to attach to the cell.

Fulvestrant blocks these receptors and stops oestrogen reaching the cancer cells. Fulvestrant also reduces the number of receptors on the breast cancer cells.

Capivasertib and fulvestrant can slow the growth of the cancer or stop it from spreading.

If you are already having another [hormonal therapy for breast cancer](#) such as [goserelin](#) (Zoladex[®] or Zoladex LA[®]) or [leuprorelin](#) (Prostap[®]), you will continue to have that with this treatment.

It is best to read this information with our general information about [cancer drug treatments](#) and the type of cancer you have.

Your cancer team will talk to you about this treatment and its possible side effects before you agree ([consent](#)) to have treatment.

More information about this treatment

This information is correct at time of publishing. But sometimes the types of cancer this treatment is used for, or treatment side effects, may change between revision dates.

You can talk to your cancer team if you want more detailed information about this treatment. Or visit the [electronic Medicines Compendium \(eMC\)](#) website, which has patient information leaflets (PIL) for individual drugs.

How capivasertib with fulvestrant is given

During your course of treatment, you will meet someone from your cancer team, such as a:

- cancer doctor
- chemotherapy nurse or specialist nurse
- specialist pharmacist.

This is who we mean when we mention doctor, nurse or pharmacist in this information.

Your cancer team will discuss your treatment plan with you. They may give you a copy of the treatment plan to take home. You usually take this treatment for as long as it is working for you and any side effects can be managed.

During your course of treatment, you will have regular blood tests. This is to check that it is safe for you to have treatment.

You will meet with a doctor, nurse or pharmacist regularly during treatment. They will talk to you about your blood test results and ask how you have been feeling.

Taking capivasertib tablets

You take capivasertib as tablets. Your cancer team will give you the tablets to take home. They may give you tablets of different strengths.

Every 7 days (one week), you take capivasertib tablets twice a day for 4 days, followed by a 3-day break with no tablets.

For example, you could take capivasertib Monday to Thursday and then not take it Friday to Sunday. This can help you remember the routine.

You keep taking capivasertib in this way for as long as you are on this treatment. Always take the tablets exactly as explained. This is important to make sure they work as well as possible for you.

Make sure you:

- swallow them whole with a glass of water

- do not chew, break or crush them
- take them twice a day, morning and evening, 12-hours apart
- take them at the same times every day
- do not drink grapefruit juice or eat grapefruit during treatment with capivasertib as this can increase side effects.

If you forget to take the tablets, you can take the missed dose up to 4 hours later. If you miss a dose by more than 4 hours, just take the next dose at the usual time. Do not take a double dose. You can contact the hospital on the 24-hour number you have been given for advice.

Other things to remember about your tablets:

- Keep them in the original packaging and at room temperature, away from moisture, heat and direct sunlight.
- Keep them safe, where children cannot see or reach them.
- If you are sick (vomit) just after taking them, do not take an extra dose. Take your next dose at the usual time.
- Do not throw away unused tablets. Return them to your cancer team at the hospital.

Having fulvestrant injections

You have fulvestrant as an injection into a muscle (intramuscular injection). You may have the fulvestrant injections at the hospital or at your GP surgery. Your GP, specialist cancer nurse or practice nurse will give you the injections. If you are unable to visit the hospital or GP surgery, a district nurse may give you the injections at home.

You will have 1 dose of fulvestrant as 2 injections, 1 in each buttock. Each injection takes 1 to 2 minutes.

You have the first 3 doses 14 days (2 weeks) apart. You will then have the injections once a month.

If there is a delay in giving the injections by 1 or 2 days, how well the treatment works should not be affected. But it is important to make sure you have your injections every month as prescribed.

Tell your doctor or nurse if you are taking any medicines to thin your blood (anticoagulants) as this can cause bruising where the injections are given.

Injection site

The skin in the area that was injected may become:

- painful
- swollen
- red, if you have white skin
- darker, if you have black or brown skin.

If this happens, let your doctor know. Painkillers may help.

About side effects

We explain the most common side effects of this treatment here. We also include some that are less common.

You may get some of the side effects we mention, but you are unlikely to get all of them. And you may have some side effects, including rarer ones, that we have not listed here.

Always tell your doctor, nurse or pharmacist about any side effects you have. They can give you:

- drugs to help control some side effects
- advice about managing side effects.

It is important to take any drugs exactly as explained. This means they will be more likely to work for you.

Serious and life-threatening side effects

Some cancer treatments can cause serious side effects. Sometimes, these may be life-threatening. Your doctor, nurse or pharmacist can explain the risk of these side effects to you.

Contact the hospital

Your doctor, nurse or pharmacist will give you 24-hour contact numbers for the hospital. If you feel unwell or need advice, you can call at any time of the day or night. Save these numbers in your phone or keep them somewhere safe.

Most common side effects

These side effects happen to 10 or more people in every 100 people (10% or more) who have this treatment.

Raised blood sugar levels

Capivasertib may raise your blood sugar levels and increase the risk of diabetes. Your doctor may need to make changes to your treatment if your blood sugar is high.

You will have regular blood tests to check this. Contact the hospital on the 24-hour number if you have these symptoms during treatment.

Signs of raised blood sugar include:

- feeling thirsty
- needing to pass urine (pee) more often than usual
- feeling more tired than usual
- feeling more hungry than usual
- losing weight.

For some people, a raised blood sugar can lead to a more serious condition. Contact the hospital on the 24-hour telephone number if you have:

- difficulty breathing
- a fruity smell on your breath
- nausea and sickness that is not getting better
- tummy pain that is not getting better or getting worse
- unusual tiredness or sleepiness

If you already have diabetes, your blood sugar levels may be higher than usual. You may need more frequent blood tests. You may also need advice and support from your GP or diabetes team about managing this.

Diarrhoea

This treatment may cause severe [diarrhoea](#). Diarrhoea means passing more stools (poo) than is normal for you, or having watery or loose stools. If you have a stoma, it will be more active than usual.

Your doctor, nurse or pharmacist may give you anti-diarrhoea drugs to take at home.

If you have diarrhoea or a mild increase in stoma activity, follow their advice about:

- taking anti-diarrhoea drugs
- how much and what type of fluids to drink
- any changes to your diet that might help.

Contact the hospital straight away on the 24-hour number if any of the following things happen:

- you have diarrhoea at night
- you have uncomfortable stomach cramps
- you have diarrhoea 4 or more times in a day
- you have a moderate increase in stoma activity
- the anti-diarrhoea drugs do not work within 24 hours.

You may need to give a stool sample, or go to hospital to have fluids through a drip or antibiotics.

Skin changes

This treatment can affect your [skin](#). If your skin feels dry, try using soap-free cleansers and unperfumed moisturising cream every day. Use a sun cream of at least SPF 30. SPF stands for sun protection factor. Cover up with clothing and a hat.

If you notice any skin changes, contact the hospital as soon as possible on the 24-hour number you have been given. Your doctor may reduce or stop your treatment until the skin changes improve.

Rarely, this treatment can cause a serious skin reaction that needs to be treated immediately in hospital.

Do not take any more of this treatment and contact the hospital straight away on the 24-hour number if you have any of these symptoms:

- a skin rash that is spreading
- blistering or peeling skin
- flu-like symptoms, such as a high temperature and joint pain
- sores on your lips or in your mouth.

Urinary tract infection

This treatment can make you more likely to get a urinary tract infection. Let your doctor, nurse or pharmacist know if:

- you have pain when you pass urine (pee)
- you need to pass urine more often than usual
- your urine is cloudy or smelly.

They may test your urine. If you have an infection, you may need antibiotics.

Feeling tired

[Feeling tired](#) is a common side effect of this treatment. Try to pace yourself and plan your day so you have time to rest. Gentle exercise, like short walks, can help you feel less tired.

If you feel sleepy, do not drive or use machinery.

Feeling sick

You may feel sick during this treatment, but this is usually mild. Your doctor, nurse or pharmacist may prescribe you anti-sickness drugs to help prevent or control sickness. Take the drugs exactly as they tell you to. [If you feel sick](#), take small sips of fluid often and eat small amounts regularly. It is important to drink enough fluids.

If you continue to feel sick, or if you are sick (vomit) 1 to 2 times in 24 hours, contact the hospital on the 24-hour number as soon as possible. They will give you advice.

Sore mouth and throat

This treatment may cause a [sore mouth and throat](#). You may also get mouth ulcers. This can make you more likely to get a mouth or throat infection. Use a soft toothbrush to clean your teeth or dentures in the morning, at night and after meals.

Contact the hospital straight away on the 24-hour number, if:

- a sore mouth or throat affects how much you can drink or eat
- your mouth, tongue, throat or lips have any blisters, ulcers or white patches.

They can give you advice, and mouthwash or medicines to help with the pain or to treat any infection. Follow their advice and make sure you:

- drink plenty of fluids
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- avoid alcohol and tobacco
- avoid food or drinks that irritate your mouth and throat.

Loss of appetite

This treatment can affect your appetite. Don't worry if you do not eat much for 1 or 2 days. But if your appetite does not come back after a few days, or if you are losing weight, tell your doctor, nurse or pharmacist. They can give you advice. They may give you food or drink supplements. Or they may suggest changes to your diet or eating habits to help.

Anaemia (low number of red blood cells)

This treatment can reduce the number of red blood cells in your blood. Red blood cells carry oxygen around the body. If the number of red blood cells is low, this is called anaemia. You may feel:

- very low in energy
- breathless
- dizzy and light-headed.

If you have these symptoms, contact the hospital straight away on the 24-hour number. You may need treatment for anaemia. If you are very anaemic, you may need a drip to give you extra red blood cells. This is called a blood transfusion.

Effects on the liver

This treatment may affect how your liver works. This is usually mild and goes back to normal after treatment ends. You will have blood tests to check how well your liver is working.

Hot flushes and sweats

Hot flushes are a common side effect of this treatment. During a flush, your neck and face may feel warm. If you have white skin, your skin may look red. If you have black or brown skin, your skin colour may deepen. Flushes can last up to 5 minutes. You might also have sweats or feel anxious or irritable during a flush.

Here are some things you can do to reduce the effects of hot flushes:

- Wear natural fabrics, such as cotton.
- Wear layers so you can remove clothes as needed.
- Use cotton sheets and layers of bedding you can remove.
- Try cooling pads or pillows to keep you cool.
- Keep rooms cool or use a fan.
- Have cold drinks and avoid caffeine and alcohol.

You might have fewer hot flushes as your body adjusts to this treatment. You could try therapies to help you [cope with hot flushes](#), such as:

- [talking therapies](#) like cognitive behaviour therapy (CBT)
- controlled breathing exercises

- yoga
- acupuncture.

If they do not improve, talk to your doctor. Certain drugs can help to improve hot flushes.

Flushes usually stop a few months after treatment ends. But some people continue to have them.

Muscle or joint pain

You may get pain in your muscles or joints. If this happens, tell your doctor or nurse. They can give you advice and painkillers or other drugs to help. Being physically active and keeping to a healthy weight may help with the pain. It can keep your joints flexible and reduce stress on them. If the pain does not get better, tell your doctor or nurse. They may talk to you about changing to a different hormonal drug.

Other side effects

These side effects happen to less than 10 in 100 people (less than 10%) who have this treatment. Some of them are much rarer than this but they are still important to know about. Rare means a side effect that happens to less than 1 in 1,000 people (less than 0.1%).

Fever

This treatment can cause a fever. Contact the hospital straight away on the 24-hour number for advice if you have a fever or a temperature over 37.5°C.

Allergic reaction

This treatment might cause an allergic reaction. Signs of a reaction can include:

- a skin rash or itching
- feeling breathless or wheezy
- swelling of your face, mouth or throat.

If you feel unwell or have any of these signs, contact the hospital straight away on the 24-hour number. Do not take any more of this treatment until you have checked with them.

Always call 999 if swelling happens suddenly or you are struggling to breathe.

Changes to your taste

Some foods may taste different or have no taste. Try different foods to find out what tastes best to you. You may also get a bitter or metallic taste in your mouth. Your doctor, nurse or pharmacist can give you advice. It might help to try:

- sucking sugar-free sour or boiled sweets
- eating cold foods
- eating sharp-tasting fresh fruit.

We have more information about [coping with changes to taste](#).

Tummy pain

You may get pain in your tummy (abdomen), or have indigestion. Your doctor, nurse or pharmacist can give you advice or treatment to help. Contact the hospital straight away on the 24-hour number if your symptoms do not go away, or get worse.

Weight loss

This treatment may cause weight loss. Let your doctor, nurse or pharmacist know if this happens. They can give you advice.

Headaches

This treatment may cause headaches. If you have headaches, tell your doctor, nurse or pharmacist. They can give you advice about painkillers that may help. Tell them if the headache does not get better, or gets worse.

Numb or tingling hands or feet (peripheral neuropathy)

This treatment may affect the nerves in your fingers and toes. This can cause numbness, tingling or pain in your hands or feet. This is called peripheral neuropathy. You might find it hard to do fiddly tasks such as fastening buttons or tying shoelaces.

If you have these symptoms, always tell your doctor, nurse or pharmacist.

Effects on the kidneys

This treatment can affect how the kidneys work. This is usually mild and goes back to normal after treatment ends. You will have blood tests to check how well your kidneys are working. Contact the hospital on the 24-hour number if you:

- have blood in your urine (pee)
- are passing less urine or peeing less often than usual.

Drinking fluids helps protect your kidneys. The advice is usually to try to drink at least 2 litres (3½ pints) of fluid each day. But follow any advice from your doctor, nurse or pharmacist about how much is right for you.

Bruising and bleeding

This treatment can reduce the number of platelets in your blood. Platelets are cells that help the blood to clot.

If the number of platelets is low, you may bruise or bleed easily. You may have:

- nosebleeds
- bleeding gums
- heavy periods
- blood in your urine (pee) or stools (poo)

- tiny red, brown or purple spots that may look like a rash – these spots can be harder to see if you have black or brown skin.

If you have any unexplained bruising or bleeding, contact the hospital straight away on the 24-hour number. You may need a drip to give you extra platelets. This is called a platelet transfusion.

Vaginal discomfort, dryness, discharge or bleeding

This treatment can cause vaginal dryness. It can also cause vaginal discomfort.

Non-hormonal creams, gels or lubricants can help with vaginal dryness and any discomfort during sex. You can buy these from a chemist. Or your doctor can prescribe them.

This treatment can also cause a thick, whitish vaginal discharge and thrush. Tell your doctor, nurse or pharmacist if you have any vaginal discomfort, discharge, soreness or itching. You might have slight vaginal bleeding for a short while:

- when you start treatment
- if you change from 1 hormonal treatment to another.

Vaginal dryness can also cause bleeding. Tell your doctor or nurse if you have any vaginal bleeding, especially if it lasts for more than a few days.

Problems with your mouth, teeth or jaw

If you are also on bisphosphonate treatment and having this treatment, contact the hospital straight away on the 24-hour number if you have any of the following:

- pain
- swelling or redness in your gums
- numbness or heaviness in your jaw
- loose teeth

Other important information

Blood clot risk

Cancer and some cancer treatments can increase the risk of a [blood clot](#). Contact the hospital straight away on the 24-hour number if you have any of these symptoms during or after treatment:

- throbbing pain or swelling in a leg or arm
- reddening of the skin in the area – if you have black or brown skin, this can be harder to notice, but the skin might become darker
- suddenly feeling breathless or coughing.

Always call **999** if you have:

- chest pain
-

difficulty breathing.

A blood clot is serious, but it can be treated with drugs called anticoagulants. These thin the blood. Your doctor, nurse or pharmacist can give you more information about preventing and treating blood clots.

Other medicines

Some medicines can affect how this treatment works or be harmful while you are having it. Always tell your cancer doctor, nurse or pharmacist about any drugs you are taking or planning to take, such as:

- medicines you have been prescribed
- medicines you buy in a shop, pharmacy or online
- vitamins or supplements
- [herbal drugs and complementary or homeopathic therapies](#)
- recreational drugs – for example, cannabis.

Alcohol

Some preparations of this treatment contain alcohol. If having alcohol is a problem for you, tell your doctor, nurse or pharmacist. Your blood alcohol level may be above the legal limit after you have the treatment. Do not drive or operate machinery for a few hours after having this treatment, even if you feel okay.

Vaccinations

Cancer doctors usually recommend that people with cancer have vaccinations for flu and [coronavirus \(covid\)](#). They may also recommend other vaccines, such as Shingrix[®] for shingles. These all help reduce your risk of serious illness from these infections. Most people can have these vaccines, including people with weak immune systems.

You should not have live vaccines if your immune system is weak. This includes if you are having or recently had chemotherapy, radiotherapy or other cancer treatments that affect your immune system. Live vaccines can make you unwell because they contain a very weak version of the illness they protect you against. There are several live vaccines, including the yellow fever vaccine.

It is important to ask your doctor, nurse or pharmacist for advice about having vaccinations. They can explain what vaccines are right for you and when it is best to have them.

Contraception

Your doctor, nurse or pharmacist will advise you not to [get pregnant](#) or [make someone pregnant](#) while having this treatment and for some time afterwards. The drugs may harm a developing baby. It is important to use contraception to prevent pregnancy. Follow their advice about:

- what types of contraception to use
- how long after treatment you should continue to use contraception.

Breastfeeding

You are advised not to [breastfeed](#) while having this treatment, or for some time after treatment ends. This is because the drugs could be passed to the baby through breast milk.

Your doctor, nurse or pharmacist can give you more information.

Fertility

Some cancer drugs can affect whether you can [get pregnant](#) or [make someone pregnant](#). If you are worried about this, it is important to talk with your doctor before you start treatment.

Medical and dental treatment

If you need medical treatment for any reason other than cancer, always tell the healthcare professional that you are having cancer treatment. Give them the contact details for your cancer doctor or cancer team so they can ask for advice.

If you have appointments with a dentist, always tell them you are having cancer treatment. Talk to your cancer team before you have any dental treatment.

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References



Visit the [electronic Medicines Compendium \(eMC\)](#) to download a Patient Information Leaflet (PIL) for more detailed information. The leaflet lists all known side effects.

[Cancer Information Development team](#)

Author

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Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

How we can help

How to print a PDF from this website

Instructions on how you can make a PDF version of this page using the print function on your browser.

Clinical Information Nurse Specialists

Our Cancer Information Nurse Specialists are dedicated cancer nurses available to talk to on our Macmillan Cancer Support Line.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help